

ABNORMAL VAGINAL BLEEDING

Give urgent attention to the patient with vaginal bleeding and any of:

- Pregnant → 159
- Recent delivery/miscarriage/termination of pregnancy → 164
- BP < 90/60
- Hb < 6
- Pallor with pulse ≥ 100, respiratory rate ≥ 30, dizziness/fatigue or chest pain

Manage and refer urgently:

- If BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.

Approach to the patient with abnormal vaginal bleeding not needing urgent attention:

- Do a pelvic examination to check for pelvic masses, a speculum examination to visualise cervix and a cervical screen if needed → 55. If abnormal, refer.
- If > 40 years, ask about menopausal symptoms: hot flushes, night sweats, vaginal dryness, mood changes, difficulty sleeping and sexual problems → 169. If new bleeding occurs > 1 year after final period, refer within 2 weeks.
- If patient is not menopausal, determine the type of bleeding problem:

Heavy or prolonged periods

- If bleeding from elsewhere like easy bruising/purple rash/bleeding gums, arrange FBC and refer to doctor next day.
- If Hb < 12, treat for likely **anaemia** → 27.
- Give COC¹: **ethinylestradiol/levonorgestrel** 30mcg/150mcg for 3 months → 154. If pregnancy desired or COC contraindicated², discuss/refer.
- Give **ibuprofen**³ 400mg 8 hourly with food for 3 days.
- If on injectable contraceptive or subdermal implant: reassure that abnormal bleeding is common in first 3 months.
- If bleeding persists > 3 months, give COC¹ or ibuprofen as above.
- Refer the patient:
 - Same week if mass in abdomen
 - If no better after 3 months on treatment
 - If excessive bleeding after IUD insertion
 - If sexual abuse suspected
 - If history of foreign body inserted into vagina

If pain during periods → 56.

Irregular periods (cycle < 21 days or > 35 days)

- If weight change, pulse ≥ 100, tremor, weakness/tiredness, dry skin, constipation or intolerance to cold or heat, check TSH. If abnormal, refer to doctor.
- Give COC¹: **ethinylestradiol/levonorgestrel** 30mcg/150mcg for 6 months → 154. If pregnancy desired or COC contraindicated², discuss/refer.

Spotting between periods

- Assess for STI → 49.
- Check Hb: if Hb < 12, treat for likely **anaemia** → 27.
- If on hormonal contraceptive, manage according to method:

Oral contraceptive:

- Ensure correct use and reassure that spotting is common in first 3 months.
- If > 24 hours diarrhoea/vomiting, advise to use condoms (continue for 7 days once diarrhoea/vomiting resolved).
- If on ART, rifampicin, phenytoin or carbamazepine, change to copper IUD or injectable → 154.
- If bleeding persists > 3 months:
 - If on progesterone-only pill and bleeding troublesome, change method → 154.
 - Switch to COC¹ containing lowest dose of ethinylestradiol (i.e. 30mcg). If bleeding persists, switch to **cypoterone/ethinylestradiol** 2mg/0.035mg daily or advise alternative method. If no better after 3 cycles, discuss.

Injectable contraceptive or subdermal implant:

- Reassure that spotting is common in first 3 months.
- If bleeding troublesome, give combined oral contraceptive (COC) **ethinylestradiol/levonorgestrel** 30mcg/150mcg. Duration depends on contraceptive method:
 - If subdermal implant, give for 20 days.
 - If on injectable, give for 14 days.
- If COC contraindicated², give instead **ibuprofen**³ 400mg 8 hourly for 3 days.

IUD:

- Reassure that spotting is common in first 3 months, is not harmful and usually resolves.
- Give **ibuprofen**³ 400mg 8 hourly with food for 3 days.

Bleeding after sex

- Assess for STI → 49.
- If assault or abuse → 88.

Refer the patient within 2 weeks if:

- Unsure of diagnosis.
- Patient complains of pelvic pain.
- Bleeding persists > 1 week after STI treatment or after diarrhoea/vomiting stop.
- Bleeding persists despite treatment.

¹Combined oral contraceptive. ²Avoid COC if smoker ≥ 35 years, migraines and ≥ 35 years old or visual disturbances, up to 6 weeks postpartum, BP ≥ 140/90, hypertension, CVD risk > 10%, blood clots, previous stroke, ischaemic heart disease or diabetes complications (eye, nerve, kidney damage). ³Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease.